

Atezolizumab + bevacizumab

Advanced or unresectable hepatocellular carcinoma / NCCP 00831 v2a

Your medicines

Atezolizumab - Bevacizumab

Your treatment schedule

Day 1 every 21 days: atezolizumab followed by bevacizumab.

A picture of side effects

Tell your team about new or worsening symptoms. Numbers link each symptom box to the body map.

Atezolizumab - immune effects

Bevacizumab - vessels / healing

1 Hormones / brain

Atezolizumab

Tiredness, thirst, dizziness or a new headache (hormone or pituitary inflammation).

3 Heart

Atezolizumab

Chest pain, palpitations or shortness of breath (myocarditis / pericarditis).

5 Kidneys

Atezolizumab

Passing less urine, swelling or blood test changes (nephritis). Bevacizumab can also cause protein in the urine, usually found on a urine test.

7 Skin / nerves / muscles

Atezolizumab

Rash or blisters; weakness, numbness or muscle pain (skin inflammation / nerve effects / myositis).

9 Blood vessels / healing

Bevacizumab

Bleeding, clots or slow wound healing (bevacizumab-related effects).

2 Lungs

Atezolizumab

New cough or shortness of breath (pneumonitis).

4 Liver

Atezolizumab

Yellow eyes, dark urine or changes in blood tests (hepatitis).

6 Bowel

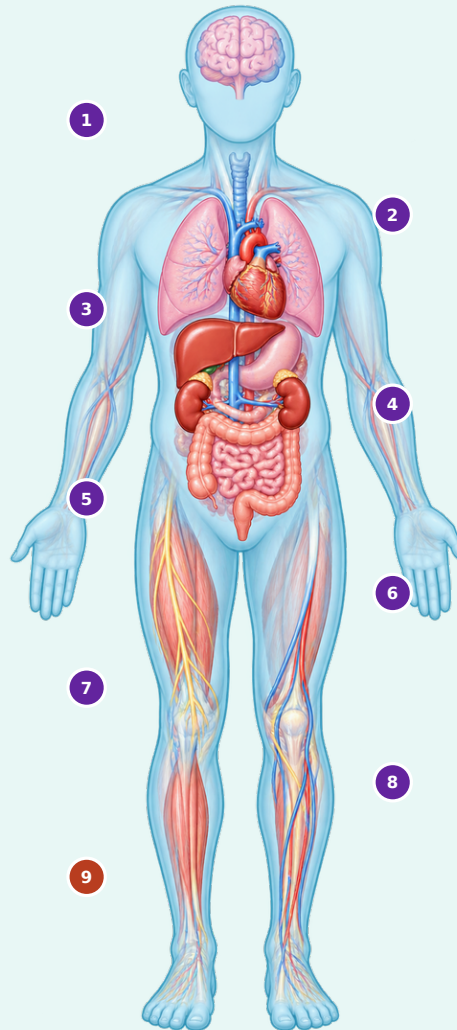
Atezolizumab

New diarrhoea, blood or tummy pain (colitis; severe pain may have another urgent cause). Bevacizumab can rarely cause a gut perforation; severe belly pain needs urgent assessment.

8 Joints

Atezolizumab

Painful, swollen joints (inflammatory arthritis).



Selected effects only. The map does not show likelihood and cannot diagnose the cause of a symptom.

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Your treatment journey

Before C1: Varices screening; blood count, liver/kidney, glucose, thyroid, BP and urine protein assessment.

C1 - Day 1: Atezolizumab IV or SC, then bevacizumab IV. Tell the nurse immediately about an infusion or injection reaction.

Days 2-21: Notice new symptoms. Follow any home BP instructions and keep your SOS number accessible.

C2 - Day 22: Repeat review, bloods, BP and urine; thyroid testing is scheduled every six weeks. Treatment only if appropriate.

Later / after: Each cycle follows the same 21-day pattern. Immune effects may still appear after treatment stops.

Monitoring and support

- Before starting Bleeding and liver context Oesophageal varices screening, BP, urine protein, blood count, liver/kidney profile, glucose and thyroid tests. Discuss past bleeding, surgery, autoimmune illness and medicines with the team.
- Each cycle Blood, urine, BP NCCP specifies blood count, liver/kidney profile, glucose and urine-protein dipstick before each cycle, plus BP before and after treatment. Results are assessed alongside symptoms.
- Every 6 weeks Thyroid tests Thyroid function tests are scheduled every six weeks. Your team may order additional endocrine tests when symptoms suggest adrenal, pituitary or glucose problems.

WHEN TO CALL

Emergency: call 112/999 for collapse, severe bleeding or vomiting blood, severe chest pain or breathing difficulty, seizure, sudden vision loss or a major neurological change. Contact oncology urgently for black stools or concerning bleeding, severe or persistent diarrhoea, new cough or breathlessness, jaundice, reduced urine, new severe headache or vision change, marked weakness or confusion, temperature ≥ 37.5 C or ≤ 36 C, or feeling suddenly unwell.

Consent discussion support

Use the approved consent process to document treatment intent, expected benefit, material risks, alternatives, questions and the patient's decision.

Scan to open the regimen-specific patient portal

<https://sactcheck.com/patient/00831/>

Official NCCP 00831 v2a

SACTCheck v0.80.4 - patient information prototype for clinical review. Follow your oncology team and local emergency instructions.

